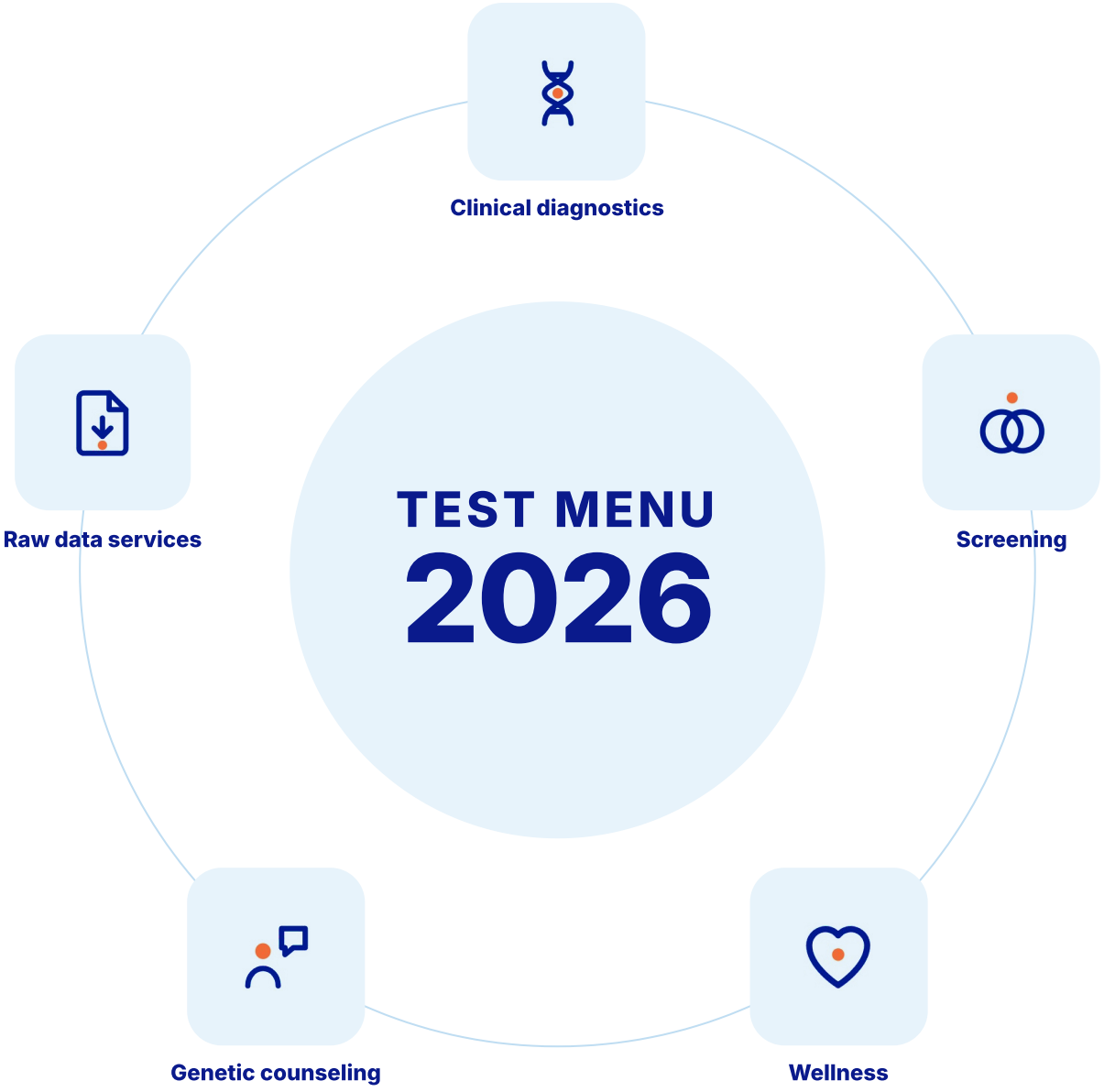


Turning Vision Into Value.

Bioscript Center by Qaly Health Shield
Clinical genomics laboratory, Dubai Silicon Oasis, UAE

WELCOME

Bioscript is a clinical genomics laboratory in Dubai. We sequence, analyze and interpret DNA in-house, turning complex genetic data into clear answers for patients, families and physicians. Every result is reviewed by specialists and written so that it can be acted on. Bioscript is part of Qaly Health Shield.

If you are a patient or a family.

Each section opens with a short explanation for you, and every test has an "In plain words" line. Your doctor or our genetic counselors will help you choose, in Arabic or English.

If you are a doctor or a partner.

Every test lists its indications, what it detects, its limitations and a "For clinicians" box with the method and standards.

CONTENTS

01	**Why Bioscript** What makes our answers different	03
02	**How it works** From sample to answer, and our standards	04
03	**Tests at a glance** All 16 test codes and turnaround times	05
A	**Clinical diagnostics** Finding the genetic cause	06
B	**Screening** Carrier, premarital and newborn screening	09
C	**Wellness** Genetics for staying well	12
D	**Genetic counseling** Understanding comes first	13
E	**Raw data services** Your genome data, in standard files	14
+	**Ask us which test is right for you** Ordering, contact and location	15

WHY BIOSCRIPT

Clear answers, close to home.

Choosing a genetic test is an important decision. Here is what every Bioscript test brings to you, your family and your doctor.

Everything under one roof.

From your sample to the signed report, sequencing, analysis and interpretation all happen in our own Dubai laboratory.

Your data stays in the UAE.

Genomic data is stored in the UAE, and you can receive your own FASTQ and VCF files as a lifelong personal record.

Read with regional insight.

Variants are interpreted with regional population frequencies from ScriptBase, so results reflect the families we serve.

Experts behind every report.

Every report is reviewed and signed by clinical scientists to international ACMG/AMP standards, with clear next steps.

Counseling in your language.

Our genetic counselors guide you before and after testing, in Arabic or English, in person or online.

A licensed clinical laboratory.

Bioscript is a DHA-licensed clinical genetics laboratory, part of Qaly Health Shield.

WHO WE WORK WITH

Patients and families
Clear answers and support at every step.

Physicians
Reports you can act on, with methods and standards stated.

Hospitals and clinics
One partner for diagnostics, screening and counseling.

Researchers
Sequencing and raw data in standard formats, with QC metrics.

16	5	16	2
test codes	service areas	specialty gene panels	languages: Arabic and English

HOW IT WORKS

From sample to answer.

From the first question to the final answer, everything happens under one roof in Dubai, with a genetic counselor at your side.

01
The right test.
Your doctor or our genetic counselor reviews your question and family history and chooses the test.

02
A simple sample.
Usually blood or saliva. For newborns, a dried blood spot.

03
Analyzed in Dubai.
Sequenced on MGI DNBSEQ-G400 and analyzed with our own pipeline by clinical scientists.

04
A clear answer.
A signed report your doctor can act on, explained to you in counseling, in Arabic or English.

Results within 29 working days. Each sample is sequenced, analyzed in full and reviewed by clinical scientists before the report is signed, so the answer you receive is one you and your doctor can rely on.

OUR STANDARDS

What every Bioscript result rests on.

- Whole exome and whole genome sequencing performed in-house on the MGI DNBSEQ-G400 platform
- Variant classification according to ACMG/AMP 2015, with ClinGen and ACGS updates
- Genetic counseling before and after testing, for patients, couples and families
- A DHA-licensed clinical genetics laboratory
- Our own analysis pipeline, from raw reads to annotated variants (GRCh38)
- Reports reviewed and signed by clinical scientists, with clear recommendations
- Genomic data kept in the UAE; patients can receive their own FASTQ and VCF files
- Reports and counseling available in Arabic and English

Meet QAly, our AI genetics assistant.
On the Bioscript website, QAly explains conditions, tests and reports in simple words, in Arabic and English, at any time. It can read a report you upload and help you prepare questions for your doctor.

TESTS AT A GLANCE

Every test code, one list.

Quote the test code when you order. Every sequencing test is reported within 29 working days, including full analysis and specialist review.

CODE	SERVICE / TEST DESCRIPTION	TURNAROUND	PAGE
A Clinical diagnostics			
WHOLE EXOME SEQUENCING (WES)			
BSC-WES-80	Standard Whole Exome Sequencing (100x)	**29 working days**	**06**
BSC-WES-TRIO	TRIO – Whole Exome Sequencing (100x): affected child, father and mother	**29 working days**	**06**
WHOLE GENOME SEQUENCING (WGS)			
BSC-WGS-30	Whole Genome Sequencing (30x)	**29 working days**	**07**
BSC-WGS-TRIO	TRIO – Whole Genome Sequencing (30x): affected child, father and mother	**29 working days**	**07**
SPECIALIZED PANELS			
BSC-WES-TP	Virtual gene panel analyzed from Whole Exome Sequencing (WES) data	**29 working days**	**08**
BSC-WGS-TP	Virtual gene panel analyzed from Whole Genome Sequencing (WGS) data	**29 working days**	**08**
B Screening			
PREMARITAL SCREENING			
BSC-WGS-PS	Premarital Screening – Whole Genome Sequencing (30x): carrier screening for couples	**29 working days**	**09**
BSC-WES-PS	Premarital Screening – Whole Exome Sequencing (100x): carrier screening for couples	**29 working days**	**09**
CARRIER SCREENING			
BSC-WGS-CS	Carrier Screening Solo – WGS (100x)	**29 working days**	**10**
BSC-WES-CS	Carrier Screening Solo – WES (100x)	**29 working days**	**10**
NEWBORN SCREENING			
BSC-WGS-NS	Standard WGS (30x) – Newborn Screening	**29 working days**	**11**
BSC-WES-NS	Standard WES (100x) – Newborn Screening	**29 working days**	**11**
C Wellness			
BSC-WGS-WL	Proactive Whole Genome Sequencing (30x)	**29 working days**	**12**
D Genetic counseling			
BSC-GC	Pre- and Post-Test Genetic Counseling	**30–45 min per session**	**13**
E Raw data services			
BSC-WGS-RD	WGS Raw Data Service (FASTQ + VCF)	**29 working days**	**14**
BSC-WES-RD	WES Raw Data Service (FASTQ + VCF)	**29 working days**	**14**

SECTION A

Clinical diagnostics.

Finding the genetic cause.

FOR PATIENTS

When symptoms may have a genetic cause, the right test can end years of uncertainty. We read the genes in our Dubai laboratory and explain the answer to you and your doctor.

FOR DOCTORS AND PARTNERS

First-tier exome and genome sequencing, singleton or trio, and specialty virtual panels on the same platform. Variants classified to ACMG/AMP with ClinGen updates; reports signed by clinical scientists. Research and population genomics projects on request.

Diagnostic whole exome.

Whole exome sequencing (WES) reads all ~20,000 protein-coding genes in one test. The exome is only about 1–2% of the genome, but it holds most of the known disease-causing variants.

SAMPLE
Blood or saliva

CODE	SERVICE / TEST DESCRIPTION	METHOD	COVERAGE	TURNAROUND
BSC-WES-80	Standard Whole Exome Sequencing (100x)	WES	**100x**	**29 working days**
BSC-WES-TRIO	TRIO – Whole Exome Sequencing (100x): affected child, father and mother	WES	**100x**	**29 working days**

IN PLAIN WORDS Complex symptoms can mean years of single tests without answers. WES is today's first-line test for many patients with a suspected genetic condition, especially when symptoms do not point to a single gene.

WHO IT IS FOR

- Developmental delay, intellectual disability and autism with other features
- Epilepsy and neurological disorders
- Multiple congenital anomalies and dysmorphic features
- Suspected metabolic, neuromuscular or immune disorders
- Newborns and children with a suspected genetic condition
- Unexplained conditions in consanguineous families

WHAT IT DETECTS

- Single-letter changes (SNVs) and small insertions or deletions in coding regions
- Splice-site variants
- Copy-number variants (CNVs): deletions and duplications of DNA
- ACMG secondary findings, if requested

LIMITATIONS Deep intronic and regulatory variants, repeat expansions (for example Fragile X) and some structural changes may be missed, as may low-level mosaicism.

FOR CLINICIANS

- Exome sequencing of ~20,000 protein-coding genes and flanking intronic regions, singleton or trio, with phenotype-driven (HPO) variant prioritization
- MGI DNBSEQ-G400, paired-end 2×150 bp; alignment to GRCh38; exome-based CNV calling
- Classification: ACMG/AMP 2015 with ClinGen SVI and ACGS updates; regional frequencies from ScriptBase
- ACMG SF v3.3 secondary findings (84 genes) on request; P/LP variants confirmed by an orthogonal method where required
- Trio analysis identifies de novo variants and confirms inheritance

Diagnostic whole genome.

Whole genome sequencing (WGS) reads almost the entire genome: coding and non-coding regions, with even coverage. It is the most complete genetic test available in one step.

SAMPLE
Blood

CODE	SERVICE / TEST DESCRIPTION	METHOD	COVERAGE	TURNAROUND
BSC-WGS-30	Whole Genome Sequencing (30x)	WGS	**30x**	**29 working days**
BSC-WGS-TRIO	TRIO – Whole Genome Sequencing (30x): affected child, father and mother	WGS	**30x**	**29 working days**

IN PLAIN WORDS Some conditions stay unexplained after other genetic tests come back normal. WGS detects the variant types found by exome sequencing and by chromosomal microarray together, and adds regions that other tests cannot reach.

WHO IT IS FOR

- Unsolved cases after exome or panel testing
- Suspected structural or copy-number changes
- Newborns and children who need the broadest answer
- Conditions with known non-coding or intronic causes
- Patients who want the most complete test in one step

WHAT IT DETECTS

- SNVs and small insertions or deletions across the genome
- Copy-number variants (CNVs) and structural rearrangements
- Deep intronic and regulatory variants
- Mitochondrial DNA variants

LIMITATIONS Some repeat expansions need a confirming specialized test; very low-level mosaicism may be missed; interpretation of non-coding variants is still evolving.

FOR CLINICIANS

- Genome sequencing of coding and non-coding regions with even coverage, detecting SNVs, indels, CNVs, structural variants and mitochondrial variants in one assay
- Platform: MGI DNBSEQ-G400, paired-end 2×150 bp; alignment to GRCh38
- Repeat expansions screened computationally; positive or borderline findings confirmed by a targeted assay
- Classification: ACMG/AMP 2015 and ACMG/ClinGen CNV standards (2020); ACMG SF v3.3 on request
- Trio analysis identifies de novo variants and confirms inheritance
- Not reliable: SMN1 exon 7 (paralogous SMN2) and FMR1 CGG sizing; these need MLPA and fragment analysis

EXOME OR GENOME

Which one first.

Exome sequencing is the first-line test for many suspected genetic conditions. Genome sequencing gives the widest view in one step, and is often chosen when an exome or panel has not found the answer.

SINGLE OR TRIO

Why test the parents too.

Testing the affected child together with both parents shows which changes are new in the child and which are inherited, which helps clinical scientists interpret the result.

Diagnostic gene panel.

A virtual panel analyzes a selected group of genes linked to one clinical problem, chosen from the patient's symptoms. It gives focused reporting with fewer incidental findings.

SAMPLE
Blood or saliva

CODE	SERVICE / TEST DESCRIPTION	METHOD	COVERAGE	TURNAROUND
BSC-WES-TP	Virtual gene panel analyzed from Whole Exome Sequencing (WES) data	WES	**100x**	**29 working days**
BSC-WGS-TP	Virtual gene panel analyzed from Whole Genome Sequencing (WGS) data	WGS	**30x**	**29 working days**

IN PLAIN WORDS When symptoms point to one organ system, a focused gene group is efficient. The exome or genome is sequenced in full and only the selected genes are reported. If the panel does not explain the symptoms, the analysis can be widened to the full exome or genome without a new sample.

WHO IT IS FOR

- A clear clinical picture pointing to one organ system
- Confirming a suspected diagnosis in a known gene group
- Family members of a patient with a known panel diagnosis

WHAT IT DETECTS

- SNVs and small insertions or deletions in all panel genes
- Copy-number changes where coverage allows

LIMITATIONS Only the selected genes are reported; repeat expansions need specialized tests.

FOR CLINICIANS

- Gene lists curated from PanelApp, ClinGen gene-disease validity and the literature; filtering restricted to panel genes
- Sequenced at the coverage of the underlying exome (100x) or genome (30x); coverage per gene available on request
- Incidental findings outside the panel are not analyzed
- Classification: ACMG/AMP 2015 with ClinGen SVI and ACGS updates

The 16 specialty panels.

TYPICAL REASONS FOR TESTING

Neurology
Seizures, developmental delay, ataxia, neuropathy

Metabolism and Endocrinology
Hypoglycemia, lactic acidosis, hyperammonemia

Ophthalmology
Cataract, nystagmus, optic atrophy, retinal dystrophy

ENT and Audiology
Sensorineural and conductive hearing loss

Craniofacial and Dysmorphology
Cleft lip or palate, craniosynostosis, facial features

Gastroenterology and Hepatology
Cholestasis, hepatomegaly, liver failure

Pediatrics
Failure to thrive, growth delay, short stature

Orthopedics and Skeletal
Skeletal dysplasia, recurrent fractures, scoliosis

Cardiology
Cardiomyopathy, arrhythmia, congenital heart defects

Dermatology
Ichthyosis, pigmentation changes, hair abnormalities

Nephrology
Nephrotic syndrome, cystic kidneys, CKD

Pulmonology
Respiratory insufficiency, recurrent infections

Neuromuscular
Muscle weakness, hypotonia, myopathy, dystrophy

Hematology
Anemia, bleeding and clotting disorders

Immunology and Multisystem
Immunodeficiency, recurrent fever and infections

Dental
Missing or malformed teeth, enamel and dentin defects

SECTION B

Screening.

Planning a family, protecting a newborn.

FOR PATIENTS

From knowing your own carrier status, to screening as a couple before marriage, to genomic screening of your newborn, these tests help families plan with confidence.

FOR DOCTORS AND PARTNERS

Individual and premarital couple carrier screening based on the exome or genome, and genomic newborn screening for treatable conditions.

Premarital couple carrier screen.

Exome- or genome-based screening of both partners for the widest view, with one shared answer.

SAMPLE
Blood, both partners

CODE	SERVICE / TEST DESCRIPTION	METHOD	COVERAGE	TURNAROUND
BSC-WGS-PS	Premarital Screening – Whole Genome Sequencing (30x): carrier screening for couples	WGS	30x	29 working days
BSC-WES-PS	Premarital Screening – Whole Exome Sequencing (100x): carrier screening for couples	WES	100x	29 working days

IN PLAIN WORDS Many healthy people silently carry a change in one gene. When both partners carry a change in the same gene, their children can inherit the condition. Screening both partners together shows this before you plan a family.

WHO IT IS FOR

- Couples planning marriage or a family
- Related (consanguineous) couples
- Couples with a family history of an inherited condition

WHAT IT COVERS

- Autosomal recessive and X-linked conditions across the exome or genome
- Shared risks reported as a couple, not two separate results
- ACMG secondary findings, on request

LIMITATIONS Spinal muscular atrophy (SMA) and Fragile X are not detected by sequencing. A negative result lowers but does not remove risk.

FOR CLINICIANS

- Carrier screening for autosomal recessive and X-linked conditions, reported per couple with shared-carrier risk, in line with the ACMG 2021 tiered approach
- Only pathogenic and likely pathogenic variants are reported; VUS are not reported for carrier screening
- SMA (SMN1 copy number) and Fragile X (FMR1) need adjunct methods (MLPA, fragment analysis)
- Residual risk depends on gene, method and ancestry; secondary findings per ACMG SF v3.3 on request

Carrier screening solo.

Know your own carrier status, whether or not you are planning to marry soon.

SAMPLE
Blood or saliva

CODE	SERVICE / TEST DESCRIPTION	METHOD	COVERAGE	TURNAROUND
BSC-WGS-CS	Carrier Screening Solo – WGS (100x)	WGS	**100x**	**29 working days**
BSC-WES-CS	Carrier Screening Solo – WES (100x)	WES	**100x**	**29 working days**

IN PLAIN WORDS Many healthy people silently carry a change in one gene. Knowing your own carrier status helps you and your family plan ahead.

WHO IT IS FOR

- Adults who want to know their own carrier status
- Relatives of someone with a known inherited condition
- Partners who will be tested at different times

WHAT IT COVERS

- Recessive and X-linked conditions, for you alone
- A broad check of many genes at once, using your exome or genome
- Your partner can be added later for a shared couple result

GOOD TO KNOW

- SMA and Fragile X are not detected by sequencing
- A negative result lowers but does not remove risk
- Our genetic counselors explain what your result means for you and your family

FOR CLINICIANS

- Carrier screening for autosomal recessive and X-linked conditions, in line with the ACMG 2021 tiered approach
- Only pathogenic and likely pathogenic variants are reported; VUS are not reported for carrier screening
- SMA (SMN1 copy number) and Fragile X (FMR1) need adjunct methods (MLPA, fragment analysis)
- Residual risk depends on gene, method and ancestry

SOLO OR AS A COUPLE

Which screen fits you.

Solo screening tells you your own carrier status. If you are planning a family together, premarital couple screening (BSC-WGS-PS or BSC-WES-PS) checks both partners and reports your shared risk in one answer.

WHY IT MATTERS

Planning with confidence.

Being a carrier rarely affects your own health. Its value is in knowing early, so that you, your partner and your doctor can make informed choices for future pregnancies.

Newborn screening.

The broadest look at a baby's genetic health, as a complement to routine newborn screening.

SAMPLE
Dried blood spot or blood

CODE	SERVICE / TEST DESCRIPTION	METHOD	COVERAGE	TURNAROUND
BSC-WGS-NS	Standard WGS (30x) – Newborn Screening	WGS	**30x**	**29 working days**
BSC-WES-NS	Standard WES (100x) – Newborn Screening	WES	**100x**	**29 working days**

IN PLAIN WORDS Some serious childhood conditions look normal at birth. Reading the baby's genes can find treatable conditions early, often before the first symptoms appear.

WHAT IT COVERS

- Sequencing of the baby's genome or exome
- Treatable conditions that start in infancy and childhood, reported with clinical guidance
- Metabolic, endocrine, blood and immune conditions

GOOD TO KNOW

- It complements, and does not replace, routine biochemical newborn screening
- SMA and thalassemia deletions are not detected by sequencing

FOR CLINICIANS

- Genomic newborn screening for treatable conditions with onset in infancy and childhood, complementing biochemical newborn screening
- Reporting limited to pathogenic and likely pathogenic variants in actionable genes
- SMA and thalassemia deletions require MLPA

FOR PARENTS

A simple start.

A dried blood spot or a small blood sample is all that is needed. No extra visits for your baby, and our team guides you through every step.

IF SOMETHING IS FOUND

You are not alone.

Every reported finding comes with clinical guidance. Our genetic counselors and your doctor explain what it means and plan the next steps together.

Screening and counseling go together.

Our genetic counselors explain screening results to parents and couples, and plan next steps with your doctor. See Pre- and Post-Test Genetic Counseling (BSC-GC) on page 13.

SECTION C

Wellness.

Genetics for staying well.

FOR PATIENTS

Know your inherited risks early and see how you respond to common medicines, so that prevention becomes personal.

FOR DOCTORS AND PARTNERS

Predictive whole genome sequencing for healthy adults: ACMG actionable genes, hereditary cancer and cardiovascular genes, and pharmacogenomics.

Proactive whole genome.

Whole genome sequencing that brings cancer, heart and actionable-gene results together in one report.

SAMPLE
Blood

CODE	SERVICE / TEST DESCRIPTION	METHOD	COVERAGE	TURNAROUND
BSC-WGS-WL	Proactive Whole Genome Sequencing (30x)	WGS	**30x**	**29 working days**

IN PLAIN WORDS Some inherited risks for cancer and heart disease give no early warning. Knowing them early can change your medical care.

WHO IT IS FOR

- Healthy adults who want to know their inherited risks
- People with a family history of cancer
- Families with cardiomyopathy, arrhythmia, early heart attacks or high cholesterol

WHAT IT COVERS

- The ACMG list of medically actionable genes (v3.3)
- Hereditary cancer genes such as BRCA1, BRCA2, PALB2 and the Lynch syndrome genes
- Heart genes for cardiomyopathy, arrhythmia and familial hypercholesterolemia
- Pharmacogenomics: how you respond to common medicines
- A clear report of what you can act on

GOOD TO KNOW Genetic risk is information, not destiny. Your results guide screening and care with your doctor.

FOR CLINICIANS

- Predictive genetic testing in healthy adults for actionable inherited risk, with management guided by NCCN, ESC and ACMG recommendations
- Genes: hereditary cancer, cardiomyopathy and arrhythmia, familial hypercholesterolemia, ACMG SF v3.3 actionable genes
- Large rearrangements (for example BRCA1/2) by CNV calling; only pathogenic and likely pathogenic variants are reported
- Cascade testing offered to relatives

SECTION D

Genetic counseling.

Understanding comes first.

FOR PATIENTS

Before a test, we help you choose the right one. After it, we explain what the result means for you and your family, in your language.

FOR DOCTORS AND PARTNERS

Pre- and post-test counseling, couple and premarital sessions, and cascade testing plans for relatives, in person or online.

Pre- and post-test genetic counseling.

A session with our genetic counselor before you decide, and again when your result is ready.

SAMPLE
None needed

CODE	SERVICE	DURATION
BSC-GC	Pre- and Post-Test Genetic Counseling	**30–45 min per session**

IN PLAIN WORDS Genetic tests differ, and the wrong test can miss the answer. Genetic reports can also be complex and hard to read alone. Our counselors make sure you choose well and understand fully.

BEFORE TESTING

- Family history and pedigree review
- Choosing the right test
- What results can and cannot tell you

AFTER TESTING

- Your report explained in plain language
- Next steps for care and screening
- Written summary for you and your physician

COUPLES AND PREMARITAL

- Carrier results explained for both partners
- Options for future pregnancies
- Sensitive, confidential and family-aware

FAMILY (CASCADE)

- Who else in the family should consider testing
- How to share results with relatives
- Testing relatives for a known family variant

FOR CLINICIANS

- Genetic counseling by our specialists, before and after testing, including risk assessment, test selection, result interpretation and cascade planning
- Pre-test: pedigree, indication, test choice, consent and incidental findings discussion
- Post-test: result disclosure, recurrence risks, management and referral

SECTION E

Raw data services.

Your genome data, in standard files.

FOR PATIENTS

Your own genome or exome data, in standard files, kept as a lifelong personal record.

FOR DOCTORS AND PARTNERS

Sequencing on MGI DNBSEQ-G400 for clinicians, laboratories and researchers, with data delivered as FASTQ and VCF (GRCh38) with QC metrics.

Raw data service.

Your genome or exome sequenced in our Dubai laboratory and delivered as raw and variant files.

SAMPLE
Blood, saliva or extracted DNA

CODE	SERVICE / TEST DESCRIPTION	METHOD	COVERAGE	TURNAROUND
BSC-WGS-RD	WGS Raw Data Service (FASTQ + VCF)	WGS	–	**29 working days**
BSC-WES-RD	WES Raw Data Service (FASTQ + VCF)	WES	–	**29 working days**

IN PLAIN WORDS Your sequencing data is a lifelong personal record. These services give you, your doctor or your laboratory the files to keep and use.

WHAT YOU RECEIVE

- FASTQ raw sequencing files, with a full quality report
- VCF file with all detected variants
- Variants called against the GRCh38 reference genome

HOW IT IS DELIVERED

- On a Bioscript-branded drive
- Or by encrypted transfer, with access through the patient portal
- Data stored in the UAE

FOR CLINICIANS

- Sequencing on MGI DNBSEQ-G400, paired-end reads; FASTQ with QC report
- Delivery of raw and processed genomic data in standard formats: FASTQ and VCF (GRCh38), with QC metrics
- Secure encrypted delivery; data stored in the UAE

Ask us which test is right for you.

ORDER A TEST

Through your doctor, through our patient portal, or by booking directly with our team. A genetic counselor can guide you before you decide.

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FOR DOCTORS AND PARTNERS

Referrals and research collaboration are available on request. Every test in this menu lists its method and standards for clinicians.

ASK QALY

Our AI genetics assistant on the Bioscript website explains conditions and tests in simple words, in Arabic and English.

PRICING

For pricing, please contact our team. Quote the test code from this menu when you ask.

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Turning Vision Into Value.

This test menu is for information. It does not replace medical advice; the choice of test is made with a physician or a genetic counselor. Test content and methods may be updated; the current specification is given on the request form and the report. © 2026 Bioscript Center, part of Qaly Health Shield.